



Policy

Rights of Carers and Other Personal Support Persons

Policy code	EMHS: 163
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Version	2.00
Functional sub group	Mental Health
Summary	East Metropolitan Health Service is committed to ensuring the rights of personal support persons – which includes parent/guardians, carers, close family members and nominated persons – are respected by the health service. This commitment includes ensuring personal support persons are made aware of their rights under the Mental Health Act 2014.
Policy owner	Executive Director Mental Health
Contact	
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NSQHS Standards (second edition)	Standard 1 Clinical Governance; Standard 2 Partnering with consumers
NSMHS Standards	2010 1.12, 3.4, 7.2, 7.3, 7.4, 7.9
Status	Active

1. Background

1.1. About this document

East Metropolitan Health Service is committed to ensuring the rights of personal support persons which includes parent/guardians, carers, close family members and nominated persons – are respected by the health service. This commitment includes ensuring personal support persons are made aware of their rights under the Mental Health Act 2014.

2. Personal Support Persons

The Mental Health Act 2014 defines “personal support person” a person as meaning (as applicable):

- their enduring guardian or guardian;
- if the person is a child – their parent or guardian;
- their nominated person;
- their carer; and
- their close family member.

Enduring Guardian and guardian have the meaning given by the
Refer to [Guardianship and Administration Act 1990](#) Section 3(1)

Parent of guardian of a child means the person who has responsibility (as defined in the family court for the child
Refer to [Family Court Act 1997](#)

Carer” means a person who is the carer of another under the
Refer to [Carers Recognition Act 2004.](#)

Nominated person means a person who is nominated under Section 273 (1) of the Mental Health Act 2010
Refer to [Mental Health Act 2014](#)

2.1. Nominated Persons

Part 16, Division 3 of the Mental Health Act 2014 introduced the concept of the “nominated person”. A nominated person assists the person who made the nomination by ensuring that the person’s rights are observed and that their interests and wishes are taken into account

A nominated person is a personal support person for the purpose of the Mental Health Act 2014

A patient is entitled to have uncensored communications with his/her nominated person, including by:

- receiving visits;
- making and receiving telephone calls;
- sending and receiving electronic communications; and
- sending and receiving mail.

This entitlement is subject to any order by a psychiatrist prohibiting or limiting the patient's right to lawful communication

Refer to [Mental Health Act 2014](#) Sections 263 and 264

2.2. Rights of Nominated Persons

Unless the patient's psychiatrist reasonably believes it is not in the patient's best interests, a patient's nominated person is entitled to be provided with information about the patient's treatment and care, including information about the following matters

- mental illness for which the patient is being provided with treatment or care;
- grounds on which any involuntary treatment order was made in respect of the patient;
- treatment and care proposed to be provided to the patient;
- any other options for the patient's treatment and care that are reasonably available
- treatment provided to the patient and the patient's response to that treatment;
- the seclusion of the patient;
- the use of bodily restraint on the patient;
- services available to meet the patient's needs.

Unless the patient's psychiatrist reasonably believes it is not in the patient's best interests, a patient's nominated person is entitled to be involved in:

- the consideration of the options that are reasonably available for the patient's treatment and care; the provision of support to the patient; and
- the preparation and review of any treatment, support and discharge plan for the patient.

A patient's nominated person is entitled to be provided with information about the patient's rights under the Mental Health Act 2014 and how those rights can be accessed and exercised.

Unless authorised to do so in another capacity, a patient's nominated person is not authorised to apply on behalf of the patient for admission or discharge by a mental health service or to make a treatment decision about the provision of treatment to the patient.

The person responsible for providing the nominated person with information, or to involve the nominated person in the matter, shall continue to make reasonable efforts to do so until it is reasonable to conclude that it is impossible for the nominated person to be provided with the information or involved in the matter.

If a psychiatrist decides that a nominated person is not entitled to be provided with information or involved in a matter because it is not in the patient's best interests, he/she must advise the nominated person of the decision and the reasons for it. This advice must be filed in the patient record and a copy given to the patient. If the nominated person requests a written confirmation of the advice, the psychiatrist will provide the written confirmation, file a copy in the patient record and give a copy to the patient.

A psychiatrist may revoke a decision that a nominated person is not entitled to be provided with information or involved in a matter. If the nominated person previously requested to be provided with the information or involved in the matter, the psychiatrist must ensure that this is done as soon as practicable and a record of the provision of the information filed and a copy given to the patient.

Refer to [Mental Health Act 2014](#) Sections 266,268, 269,270,271

3. Carers and Close Family Members

A carer of a person is a person who is that person's carer under the Carers Recognition Act 2004. A close family member of a person is any of the following persons who is not also the person's carer or nominated person, but who provides ongoing care and assistance to the person:

- a spouse or de facto partner;
- a child;
- a step child;
- a parent;
- a step parent;
- a foster parent;
- a sibling;
- a grandparent;
- an aunt or uncle;
- a niece or nephew;
- a cousin;
- if the person is of Aboriginal or Torres Strait Islander descent – any person regarded under the customary law, tradition or kinship of that person's community as the equivalent of any of the persons set out in the list above.

The Mental Health Act 2014 provides that the role of carers and close family members in the provision of treatment, care and support to a person who has a mental illness should be acknowledged and respected.

Health services must ensure that when being admitted or received, a person is asked whether they have a carer and a close family member, and whether they consent to those persons being provided with information about or involved in the treatment and care of the person.

- If the person does not consent, or the consent is given but later withdrawn, the health service must ensure that the person is periodically asked during their admission whether they consent to the information being provided to their carer or close family member.

Health services must ensure that a record is filed of each time when the person is periodically asked to give consent, and the person's answers at that time.

A person who consents to a carer and a close family member being provided with information about or involved in the treatment and care of the person can withdraw consent at any time; likewise, a person who refuses consent can consent at any time.

Refer to [Mental Health Act 2014](#) Sections 281,282, 298,296,297

3.1. Rights of Carers and Close Family Members

Subject to any limitations regarding consent, any carer or close family member of a patient is entitled to be provided with information about the patient's treatment and care, including information about the following matters

mental illness for which the patient is being provided with treatment or care;
the grounds on which any involuntary treatment order was made in respect of the patient
the treatment and care proposed to be provided to the patient;
any other options for the patient's treatment and care that are reasonably available;
the treatment provided to the patient and the patient's response to that treatment;
seclusion of the patient;
use of bodily restraint on the patient;
services available to meet the patient's needs.

Subject to any limitations regarding consent, any carer or close family member of a patient is entitled to be involved in:

- the consideration of the options that are reasonably available for the patient's treatment and care;
- the provision of support to the patient; and
- the preparation and review of any treatment, support and discharge plan for the patient.

Any carer or close family member of a patient is entitled to be provided with information about the patient's rights under the Mental Health Act 2014 and how those rights can be accessed and exercised

Any carer or close family member of a patient is entitled to be provided with information about the carer's or close family member's rights under the Mental Health Act 2014 and how those rights can be accessed and exercised.

Unless authorised to do so in another capacity, any carer or close family member of a patient is not authorised to apply on behalf of the patient for admission or discharge by a mental health service or to make a treatment decision about the provision of treatment to the patient.

Refer to [Mental Health Act 2014](#) Sections 258 (1), (3)

4. Patient Consent to Disclosure to or Involvement by Carers or Close Family Members

Whether a carer or close family member is entitled to be provided with information about the patient's treatment and care, or to be involved in matters relating to the patient's treatment and care, is dependent upon:

- the status of the patient under the Mental Health Act 2014; and
- the capacity of the patient to consent

The person responsible for providing any carer or close family member with information, or to involve any carer or close family member in a matter, shall continue to make reasonable efforts to do so until it is reasonable to conclude that it is impossible for any carer or close family member to be provided with the information or involved in the matter.

If a psychiatrist decides that a carer or close family member is not entitled to be provided with information or involved in a matter because it is not in the patient's best interests, he/she must advise the carer or close family member of the decision and the reasons for it. This advice must be filed in the patient record and a copy given to the patient. If the carer or close family member

requests a written confirmation of the advice, the psychiatrist will provide the written confirmation, file a copy in the patient record and give a copy to the patient

A psychiatrist may revoke a decision that a carer or close family member is not entitled to be provided with information or involved in a matter. If the carer or close family member previously requested to be provided with the information or involved in the matter, the psychiatrist must ensure that this is done as soon as practicable and a record of the provision of the information filed and a copy given to the patient.

Type of patient	Capacity to consent	No capacity to consent
Voluntary patient	Carer / close family member has right to information or involvement, with patient consent	Carer / close family member has right to information or involvement, unless not in patient's best
Involuntary patient or mentally impaired accused	Carer / close family member has right to information or involvement, unless patient refuses consent and such refusal is reasonable	interests Carer / close family member has right to information or involvement, unless not in patient's best interests

Refer to [Mental Health Act 2014](#) Sections 296-289,291,292,294

4.1. Explanation of Rights of Carers, Close Family Members or Other Personal Support Persons

The Mental Health Act 2014 provides that a mental health service must explain a patient's rights to them at various stages throughout the patient journey (including at admission, when an inpatient treatment order is made and when a community treatment order is made).

The person responsible for giving the explanation must also ensure that a carer, close family member or other personal support person of the patient is provided with an explanation of the patient's rights.

It is immaterial that the patient might not have consented to the carer, close family member or other personal support person being provided with that explanation of rights.

Refer to [Mental Health Act 2014](#) Section 245

5. Persons Best Interest

In deciding what is or is not in the best interest of a person, health services are to have regard to:
the wishes of the person;
the views of the person's enduring guardian or guardian, parent or guardian (if the person is a child), nominated person, carer and close family member; and
any other matter that the health service considers relevant to making the decision.

Refer to [Mental Health Act 2014](#) Section 7

6. Compliance monitoring and legislative penalties

6.1. Compliance monitoring

Each EMHS Site/Service Executive Director are responsible for ensuring staff in their area/department are made aware of and comply with this policy, and that compliance risks and training requirements are managed.

Staff are to be reminded that compliance with all policies is mandatory.

6.2. Legislative obligations

There are legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

7. Legislation

- Carers Recognition Act 2004
- Mental Health Act 2014
- [Guardianship and Administration Act 1990](#)
- [Family Court Act 1997](#)

8. Additional references

8.1. WA Health policies

Nil

8.2. EMHS policies

Nil